



Institution Name: Directorate General of Quality Assurance Centre, MoH

Document Title: Policy & Procedure of Safe Surgery: Pre-Operative
Communication

Approval Process

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Table of Contents

1. Purpose.....	3
2. Scope:.....	3
3. Policy.....	3
4. Procedure.....	3
5. Document History and Version Control.....	6
6. Related Documents :.....	6
7. Attachments.....	6
8. References.....	7



1. Purpose

Ensuring that all pre-operative criteria are met for each patient before undergoing a surgical procedure and patient information are communicated correctly when transfer to Operating Room.

2. Scope

National wide

3. Policy

3.1 It is the policy of Ministry of Health to improve effective communication and continuity of patient care through initiating standards and guidelines for documentation and safe practice within the Operating Theatre environment.

4. Procedure

- 4.1 The pre-operative checklist (see attachments) is included on the patient medical record and be completed, signed and dated.
- 4.2 The pre-operative checklist is reviewed between the nurse delivering the patient to the Operating Theatre and Operating Theatre Nurse receiving the patient into the area.
- 4.3 The pre-operative checklist remains a permanent part of the patient record.
- 4.4 The nurse accompanying the patient and the Operating Theatre staff check the patient and patient record against:-
 - 4.4.1 His/her name band. (Identification band).
 - 4.4.2 Fasting status: to prevent any episodes of regurgitation or aspiration of stomach content.
 - 4.4.3 Hematology results: to determine blood or electrolyte replacement intra-operative etc.



- 4.4.4 Teeth, dentures or known loose teeth, bridges, caps or crowns: to create awareness for the anesthetist when securing the airway during general anesthesia and prevent injury to the patient.
- 4.4.5 Allergies: To ensure that medication safety.
- 4.5 Hygiene: Patient cleanliness prevents cross infection.
- 4.6 Time of last voiding: Time of voiding is important for pelvic operations and anticipated lengthy surgery.
- 4.7 Presence of jewelry: to prevent skin damage to patient when they are moved, and to prevent burn from electro-cautery appliances.
- 4.8 Special pre-operative preparation e.g. bowel preparation: to ensure a clean surgical field.
- 4.9 Availability of blood or blood products: essential prior to commencing major surgical procedures.
- 4.10 Identification band is present and correct: to ensure correct patient receive and correct treatment.
- 4.11 Operative site is prepared: to ensure a clean surgical field free from abrasions.
- 4.12 Operative site is marked: to ensure that the correct surgical procedure is performed on the correct side.
- 4.13 Medical records are completed: to access data regarding previous complications, surgical procedure and past medical history.
- 4.14 The patient identifying himself/herself eliminates the possibility of error. Wherever possible, information regarding the patient's medical state, fasting state, allergies should be elicited from the patient directly.
- 4.15 At this time, the patient consent documentation should be checked to eliminate the possibility of errors in the operative procedure; as well as any other relevant documentation related to the patient.



- 4.16 Any discrepancies discovered at this time, should be reported to the relevant people, whether Surgeons or Anesthetist, with an Adverse Event (Incident) Reporting System.



5. Document History and Version Control

Document History and Version Control			
Version	Description of Amendment	Author	Review Date
01	Initial Release	Dr. Khaled Alhussainy Abulmajd	March/ 2020
02			
03			
04			
05			
Written by		Reviewed by	Approved by
Dr. Khaled Alhussainy Abulmajd		Concerned Team	Minister of Health

6. Related Documents:

6.1 P&P of Patient Identification (MOH/DGQAC/004/Vers 1.0).

6.2 P&P of Informed Consent (MOH/DGQAC/005/Vers 1.0).

6.3 P&P of Safe Surgery Ensure correct patient, procedure and correct site for invasive/surgical procedures (MOH/DGQAC/007/Vers 1.0).

7. Attachments:

Pre-Operative checklist



8. References:

Title of book/ journal/ articles/ Website	Author	Year of publication	Page
Care of Patient in Surgery, 12 th edition. Mosby	Roth rock, Jane C., Alexander's	2003	
Improve the accuracy of patient identification and Eliminate wrong-site, wrong-patient, and wrong-procedure	www.jacho.org	2006	
Safe patient, Safe procedure	K.A. Abulmajd	2012	
Universal Protocol for preventing wrong site, wrong procedure, wrong person surgery	The Joint Commissi on	2003	
http://www. JointCommission.org/NR/rdonlyres/DEC4A816- ED52-4C04- AF8CFEBA74A732EA/0/up_guidelnes.pdf	The Joint Commission	2006	
implementation guideline for ensuring correct patient, correct side and correct site surgery	Royal Australasian college of surgeons	2004	
Safe patient, Safe procedure	K.A. Abulmajd	2012	
AORN Position statement and correct site surgery, available at: www.aorn.org/about/positions/correctsite.htm	Association of perioperative registered nurses (AORN)	2006	
Patient Safety alert 06	NPSA	2005	



Safe Surgery Checklist

Modified from WHO—safe surgery checklist

SIGN IN (To be read out loud)

Before induction of Anesthesia

Pre-operative checklist is reviewed, completed and signed:
☐ No ☐ Yes

Has the patient confirmed his/her identity, site, procedure and consent?
☐ No ☐ Yes

Is the surgical site marked?
☐ No ☐ Yes ☐ Not applicable

Is the anesthesia machine and medication check complete?
☐ No ☐ Yes

Pre anesthesia assessment including ASA scoring and difficult airway/aspiration risk is completed :
☐ No ☐ Yes

Monitoring equipment is checked and functioning:
☐ No ☐ Yes

Risk of >500 ml blood loss (7ml/kg in children) is assessed :
☐ No ☐ Yes

Two Intra venous access in place and functioning
☐ No ☐ Yes

Responsible Name

Signature:

TIME OUT (To be read out loud)

Before start of surgical intervention for example, skin incision

Have all team members introduced themselves by name and role?
☐ No ☐ Yes

Surgeon, Anesthetists and Registered Practitioner verbally confirm:

What is the patient's name? ☐ No ☐ Yes
What procedure, site and position are planned? ☐ No ☐ Yes

Anticipated critical events explained by Surgeon:

☐ How much blood loss is anticipated?
☐ Any specific equipment requirements or special investigations?
☐ Any critical or unexpected steps to know about?

Anticipated critical events explained by Anesthetist:

☐ Are there any patient specific concerns?
☐ What is the patient's ASA grade?
☐ Any specific level of support are required

Anticipated critical events explained by Nurse:

☐ Sterility of the instruments confirmed (including indicator results)?
☐ Are there any equipment/instruments issues or concerns?

Has the surgical site infection (SSI) bundle been undertaken?

☐ No ☐ Yes ☐ Not applicable
- Antibiotic prophylaxis within last 60 minutes
Patient warming - Hair removal - Glycemic control

Has VTE prophylaxis been undertaken?

☐ No ☐ Yes ☐ Not applicable

Is essential imaging displayed?

☐ No ☐ Yes ☐ Not applicable

Surgical Implants in place?

☐ No ☐ Yes ☐ Not applicable

SIGN OUT (To be read out loud)

Before any member of the team leaves the operating room

Nurse verbally confirms with the team:

The name of the procedure been recorded?
☐ No ☐ Yes

Instruments, swabs and sharps counts are complete
☐ No ☐ Yes ☐ Not applicable

All specimens been labelled (including patient identification)?
☐ No ☐ Yes ☐ Not applicable

Surgeon, Anesthetist and Nurse:

☐ Have any equipment problems been identified that need to be assessed?

☐ What are the key concerns for recovery and management of this patient?

Patient Information

Name:

MRN:

Date: